



OFFICIAL

Antenatal Breast Expression (ABE) Guideline

1. Purpose

To fulfil the WHO and UNICEF's "10 Steps to Successful Breastfeeding" Step 6. "Give new-born infants no food or drink other than breast milk unless medically indicated" or it is mother's choice.

ABE will reduce the risk of exposing infants at risk of separation at birth from their mother, hypoglycaemia and feeding difficulties from receiving formula.

In addition, babies born to mothers with diabetes in pregnancy - either pre-existing (Type 1 or Type 2) Diabetes Mellitus (DM) or Gestational Diabetes (GDM) - are at risk of developing hypoglycaemia in the first hours after birth. Research has indicated that artificial milk contains bovine serum albumin which is associated with generating an auto-immune response. This autoimmune response has been associated with an increased risk of Type 1 DM for the infant particularly where there is also a family history of Type 1 DM. Antenatal expression of colostrum will allow storage of a small amount of colostrum which will reduce the use of artificial milk with babies at increased risk of Type 1 DM.

2. Applicability

This guideline applies to the nursing discipline of the obstetric services department within the Armadale Health Service.

3. Guidelines

Antenatal expression of colostrum will allow storage of colostrum which will reduce the use of infant formula in at risk infants.

3.1 Indications for ABE

- Women with diabetes in pregnancy
- Women who have received antenatal corticosteroids
- Women taking beta blockers (e.g Labetalol)
- Infants with antenatally diagnosed cleft lip and/or palate and congenital conditions (e.g. Downs syndrome, cardiac conditions)
- Infants of mothers having an elective LUSCS
- Established premature labour where birth is expected to proceed.
- Infants with intrauterine growth restriction.
- Women with breast hypoplasia.
- Women with hyperandrogenesis (polycystic ovarian disease)
- Women who have had breast surgery.
- Women with multiple sclerosis
- Strong family history of dairy intolerance or inflammatory bowel disease.

Before referencing this guideline document please ensure that you have the latest version from the [Policy Library](#) information hub page.

- Women who have no contraindications and wish to perform ABE.

3.2 Contraindications for ABE

- History of threatened/actual premature labour.
- Current threatened premature labour where the mother is having tocolytic/suppressive treatment
- Cervical incompetence.
- Cervical suture insitu.
- Placenta praevia type 2, 3 and 4
- History of APH

3.3 Issues for consideration

- Nipple stimulation at term may assist with cervical ripening (up to 45 mins 3 x day). However, it will only induce labour if there are sufficient oxytocin receptors in the myometrium.
- Women successfully breastfeed whilst pregnant.
- There is no significant relationship between nipple stimulation and inducing labour.
- In recognition that nipple stimulation can cause uterine contraction it is recommended that women:
 - Commence daily expression at 36 weeks gestation.
 - Start with 3-5 minutes on each breast.
 - The total time expressing when proficient should only be 5-10 mins.
 - Stop expressing if having regular uterine contractions associated with expressing.

Expressing can be commenced after admission to hospital (e.g. mother admitted in established preterm labour which is expected to proceed) unless there are contraindications as above.

3.4 Process for Education of ABE

3.4.1 Up to 32 weeks gestation

- Ensure parents are aware of the specific health benefits of colostrum and breast milk
- Encourage attendance at parent education sessions

3.4.2 32 – 35 weeks gestation

- Use information leaflets and other instructional aids to discuss the principles of breast massage and hand expression of colostrum/milk. Demonstrate the technique if applicable, encourage mother to become familiar with her breasts

3.4.3 36 weeks onwards

- Practice of technique and demonstration with the option to collect colostrum
- Supply mother with sterile containers (specimen jars/syringes with caps) for colostrum and identification labels

3.5 Patient education

3.5.1 How to hand express and store colostrum/breast milk

Antenatal expression of colostrum will increase the mother's awareness of how her breasts function and increase confidence with breastfeeding.

- Provide the woman with the pamphlet "Antenatal Expression of Colostrum". Provide 1ml-5ml syringes with caps, a feeding cup and labels.
- Wash hands before beginning.
- Ensure the woman is sitting comfortably and upright. Ensure privacy.
- Apply warmth to breast. (A warm shower. Heat packs or hot flannel's applied to the breasts)
- Gently stroke the breast towards the nipple to stimulate the let-down reflex.
- Place the fingers underneath the breast, so the first finger is just below the areola and the thumb is just above the areola, about 3-4 cm back from the nipple.
- Gently squeeze the fingers and thumb pads (not fingertips) together, back towards the chest wall into the breast tissue, and then release the pressure.
- Fingers should be well back from the nipple. Avoid squeezing or pinching the nipple.
- Repeat the action in a rhythm manner, similar to baby's sucking.
- When colostrum drips/flows easily start collecting in a syringe or feeding cup (to be syringed up at the end of expressing.)
- When the milk stops flowing, rotate the position of the fingers and thumb around the areola and repeat the expressing action to empty the entire breast.
- Change to the other breast when flow slows down.
- Use both breasts twice at each session.
- Colostrum can be collected 2-3 times on the same day and stored in the same syringe. The syringe should be refrigerated between uses.
- At the end of the collecting day, syringe up all the colostrum. Ensure the syringe is capped and labelled with name and date of expressing. Place all syringes in a zip lock bag before freezing to keep them separate from other items in the freezer.

If used, wash the feeding cup in hot soapy water, rinse thoroughly and ensure dry before the next breast expression. The frozen colostrum can be stored for 3 months in the freezer or 6-12 months in a deep freezer. Frozen breast milk may be stored as follows.

- Freezer compartment inside refrigerator 2 weeks
- Freezer compartment with separate door 3 months
- Deep freeze (-18°C or lower) 6-12 months.
- When a woman is bringing frozen expressed breast milk to the hospital, please advise her to use a high quality insulated cooler bag with ice pack to keep the milk cold.
- Advise the woman to ensure she hands the labelled frozen milk to staff as soon as they arrive at the hospital for storage.
- Store labelled colostrum in Special Care Nursery or Maud Bellas EBM freezer and communicate this to nursery staff. When removing any syringes label with the date and time of defrosting.

- Defrosted colostrum must be used within 24 hours if stored in the refrigerator or within 4 hours if stored at room temperature (less than 26°C)

3.5.2 At birth

- Place baby in skin-to-skin contact with the mother as soon as possible following birth, (if desired by the mother) and encourage mothers to recognise when their babies are ready to feed.
- Ensure first breastfeed is within the first hour following birth. If baby is not able to breastfeed; remove colostrum from breast within the first hour, either through hand expressing or with a pump.
- When baby is ready to feed offer help, if needed, to ensure good positioning and attachment.
- Monitor [blood glucose levels](#) as indicated and use fresh or defrosted colostrum if necessary.
- If baby experiencing difficulties or is uninterested in breastfeeding at this time, encourage the woman to hand express some colostrum for neonate
- If exclusive breastfeeding is not possible, establish good expressing routine to build up an abundant milk supply
- A two person check (two nurse/ midwives or mother/ nurse) must be performed prior to administration of EBM and documented on the fluid balance record by the nurse/ midwife delivering/ administering the EBM.

4. Roles and responsibilities

Role	Responsibility
Lactation Consultant	Hold an International Board-Certified Lactation Consultant (IBCLC) qualification Assist parents by providing specialist advice regarding breastfeeding and other feeding concerns.

5. Legislative context

The following documents are required to give effect to this policy:

- EMHS
 - [Patient Identification and Procedure Matching Policy EMHS: 21](#)

6. Supporting information

The following documents support the implementation of this policy:

- [Neonatal Management of Hypoglycaemia Guideline](#)

7. Compliance, monitoring and evaluation

Compliance against this guideline will be evaluated using the Baby Friendly Hospital Initiative [Self Appraisal Tool](#)

8. Relevant standards

National Safety and Quality Health Service (NSQHS) Standards (second edition):

- Standard 5 – Action 5.28: The workforce uses the systems for preparation and distribution of food and fluids to:
 - a. Meet patients' nutritional needs and requirements
 - b. Monitor the nutritional care of patients at risk
 - c. Identify, and provide access to, nutritional support for patients who cannot meet their nutritional requirements with food alone
 - d. Support patients who require assistance with eating and drinking
- Standard 6 – Action 6.5: The health service organisation:
 - a. Defines approved identifiers for patients according to best-practice guidelines
 - b. Requires at least three approved identifiers on registration and admission; when care, medication, therapy and other services are provided; and when clinical handover, transfer or discharge documentation is generated
- Standard 6 – Action 6.7: The health service organisation, in collaboration with clinicians, defines the:
 - a. Minimum information content to be communicated at clinical handover, based on best-practice guidelines
 - b. Risks relevant to the service context and the particular needs of patients, carers and families
 - c. Clinicians who are involved in the clinical handover

9. Bibliography

1. World Health Organisation. 10 Facts on Breastfeeding. 2011.
<http://www.who.int/features/factfiles/breastfeeding/en/>
2. World Health Organisation. Baby Friendly Hospital Initiative
<http://www.who.int/nutrition/topics/bfhi/en/>
3. Borsh-Johnson et al. relationships between breastfeeding and incidence rates of Insulin Dependent Diabetes Mellitus. 1984 The Lancet 10:1083-6
4. Chapman T. et al. Antenatal Breast Expression: A Critical review of the Literature. Midwifery 2013 129:203210
5. Cox, S. (2006) Expressing and storing colostrum antenatally for use in the newborn period. Breastfeeding Review. Vol 14 (3)
6. Cox S, An Ethical Dilemma: Should Recommending Antenatal Expressing and Storing of Colostrum Continue? Breastfeeding Review 2010: 18(3); 5-7
7. East C E. et al. Antenatal breast Milk Expression by Women with Diabetes for Improving Infant outcomes (Protocol) Wiley. The Cochrane Library 2003, Issue 3
8. Forster D A. et al Diabetes and Antenatal Milk Expressing: A Pilot Project to inform the Development of a Randomised Controlled Trial. Midwifery 2011:27:209-4

10. Glossary

The following definitions are relevant to this policy.

Term	Definition
EBM	Expressed Breast Milk
DM	Diabetes Mellitus

Term	Definition
GDM	Gestational Diabetes Mellitus

11. Document owner

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12. Review

This guideline will be reviewed and evaluated as required to ensure relevance and currency. At a minimum it will be reviewed within one (1) year after first issue and at least every three (3) years thereafter.

Version	Effective from	Effective to	Amendment(s)
V4	23 12 2025	23 12 2028	Full review, minor changes re patient education

13. Authorisation

This guideline has been authorised and issued by:

Authorised by	Dr Tania Strickland - Director Clinical Services
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